

1. Administrative & Study Identification

Posting ID: 61110871732948301 **Full Study Title:** A Study of Atezolizumab With or Without Tiragolumab Following Neoadjuvant Chemoradiotherapy in Participants With Locally Advanced Rectal Cancer **Official Title:** A Phase II, Randomized, Open Label, Parallel-group Study of Atezolizumab With or Without Tiragolumab Following Neoadjuvant Chemoradiotherapy in Patients With Locally Advanced Rectal Cancer **Short Title/Acronym:** NEOTERIC **Protocol Number:** ML43050 **NCT Number:** NCT05009069 **Sponsor/Company:** Hoffmann-La Roche / Shanghai Roche Pharmaceuticals Ltd China **Investigational Product:** Atezolizumab (anti-PD-L1 antibody) and Tiragolumab (anti-TIGIT antibody) **Phase of Development:** PHASE2 (Phase Requirement Met) **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting) **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy, specifically the percentage of participants with a pathologic complete response (pCR), of atezolizumab with or without tiragolumab after neoadjuvant chemoradiotherapy (nCRT) in patients with locally advanced rectal cancer (LARC). **Secondary Objectives:** To evaluate the percentage of participants with R0 resection, Objective Response Rate (ORR) before surgery, and One/Two/Three-year Relapse-free Survival (1/2/3-y RFS) rate.

2.2 Background & Rationale Neoadjuvant chemoradiotherapy (nCRT) is the standard of care for locally advanced rectal cancer (LARC), categorized under Rectal Neoplasms / Rectal Carcinoma (a neoplastic process). Pathologic complete response (pCR) rates and long-term outcomes remain suboptimal. Tiragolumab is a human monoclonal antibody targeting TIGIT. When combined with atezolizumab, it may synergistically enhance antitumor immune responses, capitalizing on the radiotherapy-induced modulation of the tumor immune microenvironment to improve pCR rates in patients with LARC.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Active Comparator (Atezolizumab alone) and historical controls **Blinding:** Open-label **Randomization:** 1:1 (Parallel-group, following an initial safety run-in phase)

3.2 Planned Enrollment & Duration Target Enrollment: 58 participants **Number of Sites:** 5 medical centers across China

Estimated Study Start: 2021 **Estimated Primary Completion:** January 6, 2025 (Data Cutoff)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Histologically or cytologically confirmed diagnosis of adenocarcinoma of the rectum. 3. Resectable locally advanced rectal cancer, with clinical stage as cT3N+M0 or cT4NanyM0 per AJCC/UICC 8th edition. 4. The inferior margin of the tumor ≤ 10 cm from the anal verge. 5. No prior anti-cancer treatment for rectal cancer. 6. Availability of a representative tumor specimen suitable for pathological evaluation and biomarker expression analysis. 7. Eastern Cooperative Oncology Group (ECOG) Performance status (PS) of 0 or 1. 8. At least one measurable lesion per Response Evaluation Criteria in Solid Tumors (RECIST) v1.1. 9. Adequate hematologic and end-organ function; stable anticoagulant regimen if receiving therapeutic anticoagulation. 10. Negative HIV test at screening. 11. Strict adherence to abstinence or contraception guidelines (for both men and women of childbearing potential). 12. Eligible to receive long-course neoadjuvant chemoradiotherapy.

4.2 Exclusion Criteria 1. Evidence of metastatic disease or presence of synchronous colorectal cancer. 2. Histology consistent with small cell carcinoma, squamous carcinoma, or mixed carcinoma. 3. Presence of obstruction, imminent obstruction, clinical symptoms, or radiological suspicion of bowel perforation. 4. Not eligible for long-course radiotherapy. 5. History of malignancies other than rectal cancer within 3 years prior to screening (excluding those with negligible risk). 6. Active or history of autoimmune disease, immune deficiency, significant cardiovascular disease, or severe chronic/active infection within 4 weeks. 7. History of idiopathic pulmonary fibrosis, organizing pneumonia, drug-induced pneumonitis, or active pneumonitis on CT. 8. Treatment with therapeutic oral or IV antibiotics within 2 weeks prior to initiation. 9. Uncontrolled tumor-related pain, uncontrolled effusions/ascites requiring recurrent drainage, or symptomatic hypercalcemia. 10. Uncontrolled diabetes or Grade ≥ 2 abnormalities in potassium or sodium despite standard medical management within 14 days prior to initiation. 11. Prior treatment with CD137 agonists, T-cell co-stimulating, or immune checkpoint blockade therapies (including anti-CTLA-4, anti-PD-1, anti-PD-L1, and anti-TIGIT). 12. Treatment with systemic immunostimulatory agents, systemic immunosuppressive medication, or live, attenuated vaccine within 4 weeks. 13. Major surgical procedure or significant traumatic injury within 28 days, or abdominal surgery/interventions within 60 days. 14. Treatment with any other investigational agent within 28 days. 15. History of severe allergic anaphylactic reactions to chimeric/humanized antibodies; allergic reactions to chemotherapy drugs (5-FU and capecitabine) or Known DPD deficiency. 16. Pregnant or breastfeeding.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **nCRT Phase (Weeks 1-5):** Long-course radiotherapy to the pelvis on Days 1-5 every week, with concurrent oral Capecitabine or fluorouracil (ATC Code: L01BC02; Trade names: Carac, Efudex, Adrucil, Fluoroplex, Tolak) 5 days/week. * **Immunotherapy Phase (Weeks 8, 11, and 14):** Atezolizumab combined with or without Tiragolumab administered on Day 1 of each 21-day cycle for 3 cycles. * **Surgery:** Radical surgery performed 2 weeks after the final immunotherapy dose. **Route of Administration:** Oral (Capecitabine), Intravenous (fluorouracil/Atezolizumab/Tiragolumab), and Pelvic Radiation.

5.2 Concomitant & Prohibited Therapy Permitted: Standard antiemetic and symptom management medications. **Prohibited:** Systemic immunostimulatory agents, systemic immunosuppressive medications, live/attenuated vaccines within 4 weeks, therapeutic antibiotics within 2 weeks.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	-28 to 0	Informed Consent, Medical History, Tumor Staging, Baseline Labs, HIV Test
2	Weeks 1-5	Pelvic Radiotherapy (Days 1-5 weekly) + Capecitabine / fluorouracil (5-FU)
3	Weeks 8, 11, 14	IV Atezolizumab ± Tiragolumab (Day 1 of 21-day cycles)
4	Week 16	Radical resection (approx. 2 weeks post-final immunotherapy dose)
5	Post-Surgery	Assessment of pCR, R0 Resection, RFS, and safety monitoring

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Percentage of Participants With Pathological Complete Response (pCR). **Measurement Method:** Central/Local pathological examination of the surgically resected tissue following Total Mesorectal Excision (TME).

7.2 Secondary Endpoints * Percentage of Participants With R0 Resection. * Objective Response Rate (ORR) Before Surgery. * One/Two/Three-year Relapse-free Survival (1/2/3-y RFS) Rate.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring of treatment-related adverse events during the nCRT and immunotherapy phases, extending into the post-operative period. **Serious Adverse Events (SAEs):** Grade 3 to 4 AEs related to study drugs reported and monitored promptly. **Safety Run-in:** Study includes a safety run-in phase prior to the randomization phase.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for survival outcomes; Safety Set for evaluating treatment-related adverse events. **Sample Size Justification:** Target enrollment of 58 participants. **Handling of Missing Data:** Standard survival analysis metrics utilized for time-to-event secondary endpoints.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study was conducted in accordance with the principles of the Declaration of Helsinki and Good Clinical Practice (GCP). **Monitoring Plan:** Supervised by Roche Global Clinical Operations. **Audit Trail:** Protected clinical data environments with informed written consent recorded from all participants.

11. Study Identifiers & Governance

Primary ID: ML43050 **Posting ID:** 61110871732948301 **Registry Number:** NCT05009069 **Sponsor/Lead Organization:** Roche **Study Title:** NEOTERIC **Official Regulatory Title:** A Phase II, Randomized, Open Label, Parallel-group Study of Atezolizumab With or Without Tiragolumab Following Neoadjuvant Chemoradiotherapy in Patients With Locally Advanced Rectal Cancer

12. Clinical Framework (Oncology Specific)

Primary Condition: Rectal Neoplasms / Rectal Cancer (Locally Advanced Rectal Cancer) **Disease Area / Semantic Type:** Neoplastic Process **Diagnosis Threshold:** Adenocarcinoma of the rectum staged as cT3N+M0 or cT4NanyM0 with inferior margin ≤ 10 cm from anal verge. **Medical Methodology:** Neoadjuvant Chemoradiotherapy (nCRT) followed by Dual Immune Checkpoint Blockade **Optimization Target:** Pathologic Complete Response (pCR) prior to surgical resection

13. Study Procedures & Methodology

Management Pathway: Standard LARC neoadjuvant downstaging pathway combined with TIGIT/PD-L1 checkpoint inhibition. **Education Protocol (Patient):** Informed consent detailing combined radiation, oral/IV chemotherapy (e.g., fluorouracil), and experimental immunotherapy risks, as well as strict contraception requirements. **Quality Audit Mechanism:** Pathological evaluation and biomarker expression analysis.

14. Observation & Follow-Up Schedule

Total Duration: Multi-year observation covering 1/2/3-year RFS milestones. **Onsite Visit Cadence:** Weekly during nCRT; Every 3 weeks during immunotherapy. **Checklist Review Interval:** Restaging

assessment and surgical planning at Week 16; ongoing RFS assessments post-surgery.

15. Sign-off & Approval

Role	Name	Signature	Date	:	---	:	---	:	---	:	---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]	:	---	:	---	:	---	:	---
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]	:	---	:	---	:	---	:	---
Statistician	[Name]	_____	[DD-MMM-YYYY]	:	---	:	---	:	---	:	---